Physicians Caring for Texans

February 23, 2026

Thomas Keane, MD, MBA
U.S. Department of Health and Human Services
Assistant Secretary for Technology Policy and the Office of the National Coordinator
HHS Health Sector AI RFI, Mary E. Switzer Building; Mail Stop 7033A
330 C Street SW
Washington, DC 20201

RE: HHS Health Sector AI RFI

Submitted Via *Federal Register*

Dear Dr. Keane,

On behalf of the Texas Medical Association (TMA) and our more than 60,000 physician and medical student members, we thank the Assistant Secretary for Technology Policy and the Office of the National Coordinator for the opportunity to respond to the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

TMA is a private, voluntary, non-profit association and the largest state medical society in the nation. It was founded in 1853 to serve the people of Texas in matters of medical care, prevention and cure of disease, and improvement of public health. Today, its vision is "improving the health of all Texans."

TMA understands the importance of proceeding carefully related to how artificial intelligence (AI) is developed, implemented, and used in health care.

TMA distinguishes between the terms "augmented intelligence" and "artificial intelligence." We agree with the industry view that artificial intelligence replaces human function and intelligence such as a self-driving car or a computer-generated chatbot on a website. Augmented intelligence provides prompts and information to support human decision-making rather than replace it. We believe that is an important distinction. Augmented intelligence is intended to co-exist with human decision-making – it should not be used to replace physician reasoning and knowledge and should neither replace nor diminish the patient-physician relationship. Throughout the letter, "AI" is used for brevity, but we ask that you keep in mind TMA's preferred terms.

TMA supports the use of augmented intelligence when used appropriately to support physician decision-making, enhance patient care, and improve public health. Augmented intelligence should also be used in ways that reduce physician burden and increase professional satisfaction.

At the same time, sufficient safeguards should be in place to assign appropriate liability inherent in augmented intelligence to the software developers and not to those with no control over the software content and integrity, such as physicians and other users.

RFI Question 1. What are the biggest barriers to private-sector innovation in AI for health care and its adoption and use in clinical care?

TMA Response: There must be trust that the AI tools are developed transparently and that data accuracy, patient safety, and clinical outcomes are prioritized during all phases of product development. This includes the ability to easily access and assess algorithms, algorithm provenance, training data assumptions, training data source(s), and performance characteristics.

AI tools used for clinical care need seamless integration with electronic health records (EHR) and other technology tools used by physicians so precise patient information is used to inform the decisions returned by AI tools. One barrier is some EHR vendors are developing their own AI tools and may choose to prevent other clinically validated tools from integrating with their proprietary EHR technology.

The initial investment can be high and is a concern and barrier when adopting new technologies in medical practice. Before savings are realized from the adoption of a technology, the investment must be made, the technology implemented, the technology outputs validated, and finally the technology optimized. Once optimization occurs, cost savings may then be realized in reduced human resources or improved data quality output resulting in practice efficiencies.

RFI Question 2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

TMA Response: Developers and users need regulatory clarity and consistency when developing and implementing AI.

There is uncertainty around disclosure and consent and when they apply, which is based on how the AI tools and vendors are using patient data. Additionally, there is no clear guidance on how to disclose or obtain consent when a physician orders diagnostic testing for the patient and it is unknown if and how a pathologist or radiologist may use AI when interpreting tests. It would also be helpful to have vendor transparency around HIPAA claims and stated HIPAA compliance as well as compliance with any other applicable regulations.

Mandating specific consents for specific AI technologies will become increasingly difficult as the number of solutions increase. Having disparate federal and state requirements will prove challenging to AI users in clinical care.

As the use of AI in health care becomes ubiquitous, there are new ways to protect patient data. Physicians and other health care covered entities are increasingly incorporating large language models (LLMs) into their practices. With AI tools, insights from the LLMs can be used for retrospective and prospective analysis of patient outcomes and physician performance. The information generated in AI tools using patient data should be protected and considered privileged and confidential and should not be used to penalize physicians.

Additionally, there should be a safe-harbor framework for physicians and other clinicians using HHS-recognized or validated tools as intended and when the physician documents appropriate review of AI-generated data.

RFI Question 3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

TMA Response: Use of AI, machine learning, and clinical decision support has inherent risks. Legal and ethical responsibility for the use and output of these products must be assumed by, including but not limited to, developers, distributors, and users with each entity owning responsibility for its respective role in the development, dissemination, implementation, and use of products used in clinical care. Many times, the end-user licensing agreements presented by technology vendors do not give options for negotiation, and the vendors fully indemnify themselves from algorithmic errors or other issues that may arise. It would be helpful to prevent this type of indemnification that undermines shared responsibility.

Physicians need protection from medico-legal liability when AI tools have insufficient training data, incorrect information in the training model, or biases in the data used to train the model that could produce faulty outputs. To increase user confidence, AI tools must have some reliability and reproducibility.

AI tools should never independently and unilaterally make decisions about a patient's diagnosis or care. There should also be shared responsibility with AI developers and deployers of AI that the products have some level of reliability and reproducibility. Just as EHRs undergo a certification process to verify the product performs certain functions, AI tools used in clinical care could undergo a similar verification process.

Once implemented, all AI tools should have a feedback loop that allows users an easy way to report potential safety hazards as well as problems and malfunctions. This feedback loop can also serve as an opportunity to recommend AI tool improvements.

RFI Question 4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care?

TMA Response: AI algorithms must be designed for the benefit, safety, and privacy of the patient.

Algorithms and other information used to derive the information presented as AI to physicians and other clinicians should:

- Be developed transparently in a way that is accessible, explainable, and understandable to clinicians and patients and that details the benefits and limitations of the clinical decision support, and/or augmented intelligence;
- Have reproducible and explainable outputs;
- Function in a way that promotes health equities while eliminating potential biases that exacerbate health disparities;

- Use best practices for a user-centered design that allows for efficient and satisfactory use of the technology;
- Safeguard patient information by employing privacy and security standards that comply with HIPAA and state privacy regulations;
- Have a feedback loop that allows users who identify potential safety hazards to easily report problems and malfunctions and that allows for opportunities to report methods for improvements; and
- Contain a level of compatibility to allow use of information between hardware and software made by different manufacturers.

It would be helpful to assess AI algorithms with standardized disclosures that include intended use, limitations, known failure modes, and independent evaluation pathway. Additionally, when available, vendors should disclose to their users the results of third-party evaluations AI vendors have undergone, which could include data on accuracy, known limitations, and, if applicable, the populations or settings in which the tool was validated. There should be an expectation of post-deployment accountability, including mechanisms to monitor the AI performance over time, identify data drift or bias, and provide clear feedback and remediation pathways to users when issues arise. Other useful metrics include workflow impact, time saved, error severity, and downstream clinical decisions.

RFI Question 5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

TMA Response: Accreditation and certification should be a carefully considered pathway. While it does instill confidence in users, it also has the potential to slow innovation. At the very least, HHS should consider required standardized reporting by AI vendors that includes information related to intended use of technology, training data sources and limitation, validation settings, and known failures modes. This information could be displayed in a standardized way similar to what is required for nutritional labels on food products.

RFI Question 6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

TMA Response: Section 1557 of the Affordable Care Act, HHS, and the Office of Civil Rights mandates the availability of qualified interpreters for patients with limited English proficiency or disabilities. Accessing interpreters can be cost prohibitive for some practices and can cost more than the payment for the service provided. Cost savings can be accrued if virtual and digital interpreters are permitted, as long as validation exists for translation accuracy.

To gain insight, TMA surveyed its members in 2025 and asked respondents how they are currently using or wanting to use AI in their practice (see figure 1). Physicians are using tools that reduce administrative burden but want to use tools for clinical decision support and quality improvement.

The use of ambient scribes has proven to be a successful use case of an AI tool that reduces administrative burden and assists physicians by allowing them to focus on the patient rather than the computer. Physicians also desire AI tools that can assist with prior authorizations and appeals (while respecting the need for physician clinical judgment), as prior authorization requirements have become so administratively burdensome to practices that additional staff are needed to handle the extra work. Physicians are still hesitant, however, to use clinical decision support tools because of the lack of validation and potential for data drift. Without some medico-legal protections, use of clinical decision support tools will continue to be hampered.

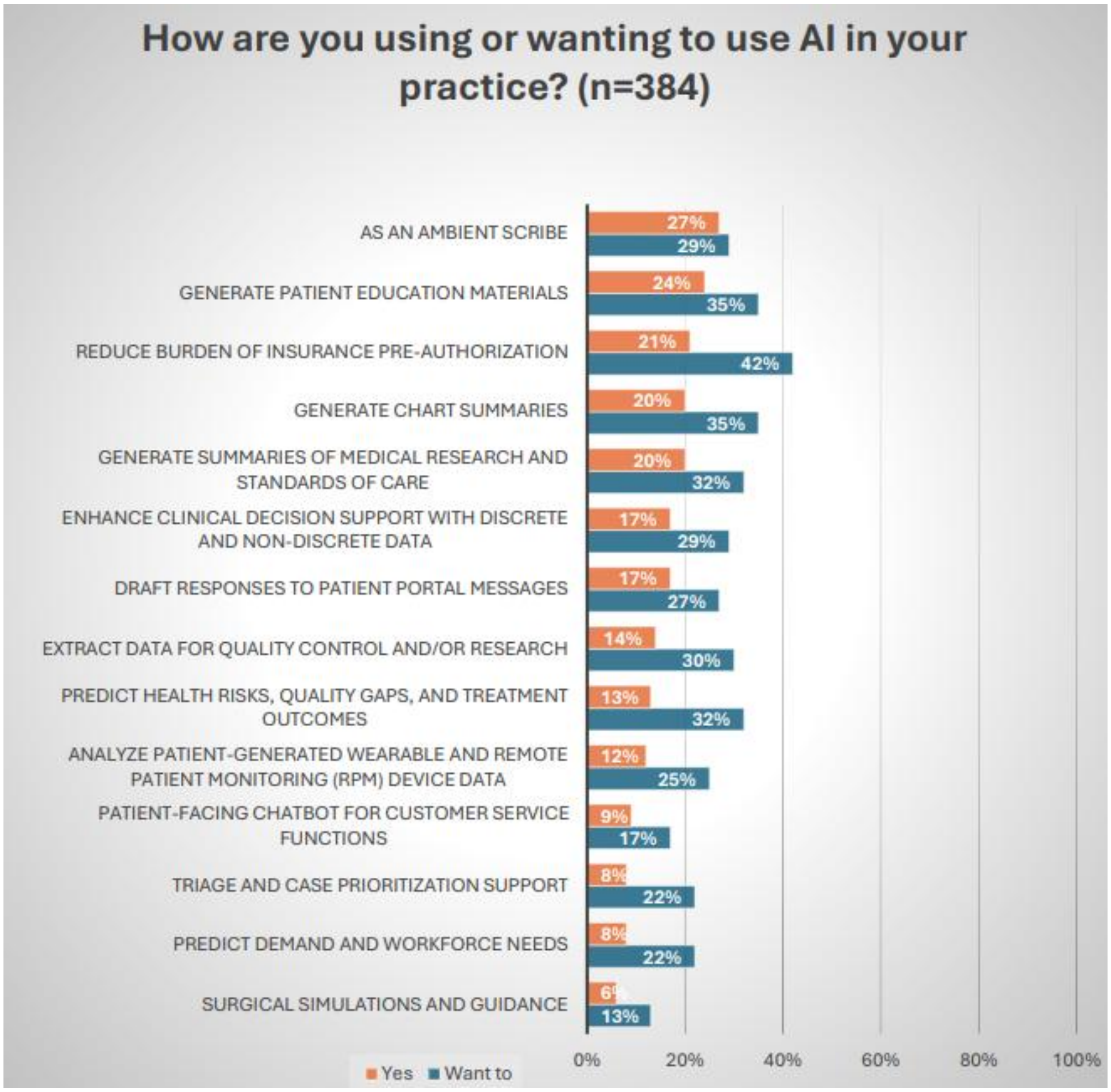

Figure 1

RFI Question 7. Which role(s), decision-maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

TMA Response: Because of the wide variety of health care entities, the roles of the decision-makers vary greatly. For example, in a small independent practice, the physician and office manager will wear many hats. In a large integrated multi-specialty practice, there may be a chief medical information officer and a chief technology officer leading the decision-making process on all technology tools, including AI, used by the organization. Some of the primary hurdles to adoption include technology costs, uncertain return on investment, time spent training users, user acceptance, validated data outputs, patient acceptance, and proven clinical outcome improvements.

RFI Question 8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

TMA Response: Interoperability remains challenging due to a lack of data vocabulary standardization. For example, in proposed United States Core Data for Interoperability (USCDI) version 6, there are a total of 132 data elements of which 57 have no correlating vocabulary standard. Interoperability continues to be hindered without standardized vocabulary for data elements. Additionally, there is a lack of fidelity and usability of the data exchanged. Interoperability should be not only about the movement of the data, but also the quality and usability of the data received. Recipients need well-organized and meaningful data outputs to efficiently analyze and make decisions about patient care. AI solutions could be developed and deployed to translate non-standardized data elements and to usefully summarize patient data, potentially according to specialty-specific specifications, enhancing the quality of the patient information received and reviewed.

RFI Question 9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

TMA Response: Physicians and patients remain concerned about unreliable technology and its use of data that may be leveraged against physicians and patients. The trust of the company and the reliability of the technology are paramount. There is also concern over how patient data and AI-generated outputs are used downstream for patient and physician analytics or other secondary purposes with unintended consequences.

A common goal should be well-developed and well-implemented AI technology that functions securely and accurately behind the scenes, optimizing information at the point of care without being intrusive to the physician or the patient.

Additionally, there is an explicit distinction between AI used for quality improvement, patient care, and clinical research versus AI used for enforcement or billing oversight. Physicians may be hesitant to engage with tools that generate insights that could later be used punitively. Physicians who choose to use AI tools for patient care must have reassurances that any oversight mechanisms installed or usage data compiled will be used for education and not to penalize its users.

RFI Question 10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

TMA Response: HHS could consider the following areas of research to accelerate the adoption of AI as part of clinical care:

- How AI tools perform after deployment, including data drift, real-world safety issues, and practical ways physicians can report concerns, near misses, or incorrect information.
- Human factors and workflow impact: When AI reduces burden versus when it unintentionally adds cognitive load or documentation work.
- Medico-legal and accountability frameworks that protect patient safety while giving physicians reasonable reliance protections when using clinically validated tools.
- Equity and bias evaluation in clinical environments, including rural settings, safety-net populations, and differing documentation practices.
- Interoperability and data-quality research that focuses not just on moving data, but on making it usable and actionable at the point of care.
- Education and competency development for trainees and practicing physicians to support safe, supervised, and appropriate use of AI in clinical care.
- Impact on medical education and whether AI degrades diagnostic reasoning or can be used as a supervised teaching tool.
- Validating patient-facing AI for clinical triage and guidance to the right level of care to assist patients struggling to navigate the health care system.
- Algorithmic vigilance systems that detect hallucinations, bias, or unsafe outputs before making clinical recommendations.
- Validation of studies in the use of AI for pattern recognition and clinical work, e.g. retinal screening and image reading.
- Small-data AI for rare diseases and specific populations.

Thank you for the opportunity to respond to the request for information. TMA stands ready to provide you and others within the agency with our policy insight and any additional assistance you may find useful. If you have any questions, please do not hesitate to contact Shannon Vogel, associate vice president of health information technology, by emailing shannon.vogel@texmed.org or calling (512) 370-1411.

Sincerely,

Jayesh "Jay" Shah, MD
President
Texas Medical Association